

JOHNS HOPKINS MEDICINE
NEUROLOGY REFERRAL INTAKE (FAX RECEIVED)

Timestamp: 04/18/2026 14:12 EST

Fax ID: 993-44-221

Pages received: 17 (some pages skewed)

Referral Reason: “progressive weakness ?neuropathy vs motor neuron disease”

Requested Service: General Neurology (auto-filled)

Priority: Routine (but “declining function” handwritten)

Referring Office: Cedar Ridge Internal Medicine

Referring Provider: M. Kline, DO

Phone: (703) 555-8821

Fax: (703) 555-8833

Patient Name: HARRIS, JONATHAN P

DOB: 09/14/1968

Insurance: BCBS PPO (card copy faint)

Notes: “pls eval for ALS?? vs myopathy. fatigable?? unclear”

Page 1 of 17

PATIENT DEMOGRAPHICS

Printed: 04/17/2026 09:02

Name: Jonathan Paul Harris

MRN: CRIM-448221

DOB: 09/14/1968 Age: 57

Sex: M

Address: 14208 Larkspur Ct, Centreville VA 20120

Phone: 571-555-1190 (cell)

Emergency Contact: spouse (illegible, possibly “Karen”)

Primary Insurance: BCBS

Secondary: none listed

Preferred Pharmacy: CVS Centreville

Employment: IT consultant (states “mostly desk work”)

Social Hx:

Tobacco: denies

Alcohol: occasional (1–2 beers weekends)

Drugs: denies

Family Hx:

Father: CAD, deceased 72

Mother: DM2

No known neuromuscular disease (one note says “uncle w/ Parkinson’s??”)

Page 2 of 17

REFERRING PROVIDER NOTE

Cedar Ridge Internal Medicine

Date: 04/16/2026

CC: progressive weakness

HPI:

57yo male with ~6–8 mo history of worsening weakness. Initially noted difficulty climbing stairs and getting out of low chairs. Over past 2 months also reports arms “give out” when lifting overhead (placing items on shelves). No clear sensory loss. Occasional shortness of breath when exerting. Denies significant pain.

Patient reports symptoms worse later in the day. Wife notes “he looks fine in the morning but by evening his shoulders slump.”

No dysphagia per patient but later in visit states “food gets stuck sometimes.”

No bowel/bladder issues.

ROS:

+fatigue

+intermittent ptosis? (patient unsure, wife noticed “droopy eyelid once or twice”)

-denies numbness/tingling

Exam:

Vitals stable

Neuro:

CN: grossly intact (no obvious ptosis today)

Motor: proximal weakness 4-/5 shoulders/hips, distal 5/5

Reflexes: 2+ symmetric

No fasciculations noted today

Gait: slow, rises from chair using hands

Assessment:

Progressive proximal weakness – concerning for possible motor neuron disease vs inflammatory myopathy vs other neuromuscular disorder. Some fatigue component but unclear.

Plan:

Refer to neurology (Hopkins requested by patient)

Labs pending

Consider EMG

Handwritten addendum: “fatigability??”

Page 3 of 17

PRIOR NOTE 1

Urgent Care Visit

Date: 11/22/2025

Chief Complaint: “weak legs”

HPI:

Pt reports 2–3 months of leg weakness. Worse with activity. Says he can walk but stairs difficult. No trauma. No back pain.

Denies numbness.

Exam:

Strength 5/5 documented (unclear)

Gait normal (contradicts pt report)

Assessment: deconditioning vs viral syndrome

Plan: rest, fluids

(Stamped) Provider: L. Chen, PA-C

Page 4 of 17

PRIOR NOTE 2

Cedar Ridge Internal Medicine

Date: 01/10/2026

CC: fatigue and weakness

HPI:

Patient returns with persistent weakness. Now includes shoulders. Reports “arms get tired brushing teeth.” Also notes occasional double vision at night (states “maybe just tired”).

Denies swallowing issues (in this note).

ROS:

+fatigue

+intermittent blurry vision

Exam:

Motor: 4+/5 proximally, 5/5 distally

Reflexes normal
No atrophy

Assessment:
Possible polymyositis vs endocrine issue

Plan:
Check CK, TSH
Consider rheumatology

Page 5 of 17

PRIOR NOTE 3
Rheumatology Consult
Date: 02/28/2026

Reason: evaluate for inflammatory myopathy

HPI:
Progressive proximal weakness. No rash. No joint swelling. Reports fatigue worsening throughout day.

Patient inconsistently reports ocular symptoms ("sometimes blurry, sometimes double").

Exam:
Strength 4/5 deltoids, hip flexors
No rash
No synovitis

Impression:
Low suspicion for classic inflammatory myopathy (CK normal previously). Consider neuromuscular junction disorder vs less likely motor neuron disease.

Plan:
Recommend neurology evaluation
ANA panel sent

Provider: S. Patel, MD

Page 6 of 17

LAB RESULTS (compiled)
Printed 04/17/2026

CK: 78 (normal)
TSH: 2.1

ANA: negative
ESR: 12
CRP: 1.2

CBC: normal
CMP: normal

AChR antibody: not done
MuSK: not done

Vitamin B12: 410

Misc:
Lipid panel elevated LDL 162
A1c 5.9

Page 7 of 17

ADDITIONAL LAB (outside)
Date: 03/05/2026

ANA panel repeat: negative
Myositis panel: negative

Page 8 of 17

IMAGING REPORT

MRI Brain without contrast
Date: 03/12/2026

Findings:
No acute infarct
No mass
Mild chronic microvascular changes

Impression:
Unremarkable for acute pathology

Page 9 of 17

MRI CERVICAL SPINE
Date: 03/12/2026

Findings:

Mild degenerative disc disease C5–C6
No significant cord compression

Impression:

Mild spondylosis

Page 10 of 17

MEDICATION LIST (as of 04/16/2026)

Atorvastatin 20 mg daily
Lisinopril 10 mg daily
Omeprazole 20 mg daily
Vitamin D 2000 IU daily
Ibuprofen PRN
Multivitamin

Note: patient stopped statin briefly “no change in symptoms”

Page 11 of 17

DISCHARGE SUMMARY (ED VISIT)

Date: 03/20/2026

Chief Complaint: shortness of breath

HPI:

Patient presented with dyspnea after climbing stairs. No chest pain.

Workup:

CXR normal
Troponin negative

O2 sat 96% RA

Exam:

Mild weakness noted but not focal

Disposition:

Discharged home

Diagnosis: exertional dyspnea

Comments: “possible deconditioning”

Page 12 of 17

PROCEDURE NOTE

Pulmonary Function Test (office spirometry)

Date: 04/02/2026

FVC: mildly reduced

FEV1: normal

FEV1/FVC: normal

Interpretation: possible restrictive pattern

Technician note: "patient fatigued during test"

Page 13 of 17

PHONE ENCOUNTER LOG

04/10/2026

Spouse called reporting worsening weakness by evening, difficulty lifting arms to wash hair.

04/12/2026

Patient reports "voice gets softer at night"

Page 14 of 17

SCANNED HANDWRITTEN NOTE (partially legible)

"...no fasciculations seen... strength variable... repeat exam after rest slightly improved??"

"...consider NMJ issue..."

Page 15 of 17

VACCINATION RECORD (partial)

Flu shot 2025

COVID booster 2024

Page 16 of 17

FAX COVER (duplicate copy)

TO: Johns Hopkins Neurology

RE: Harris, Jonathan

Reason: weakness / r/o ALS

“pls evaluate, progressive course unclear etiology”

Page 17 of 17